

CognaSync

Mode C — Provider Clinical Summary · Sample output for review (hypothetical patient, redacted)

Patient	P-2284 (redacted)	Reporting period	14 days ending May 15, 2026
Check-ins logged	12 of 14 days	Most recent entry	May 14, 2026 (1 day ago)

TRAJECTORY

Stability scores trended modestly downward across the 14-day period, with sleep disruption and self-reported headaches emerging as the most consistent co-occurring signals.

QUANTITATIVE SUMMARY

Variable	Average	Trend	Statistical detail	Range / context
Mood	5.1 / 10	declining	slope -0.18/day, R ² =0.31, p=0.04	range 3–7
Stress / Anxiety	6.4 / 10	rising	slope +0.15/day, R ² =0.22, p=0.09	range 4–8
Sleep	6.2 hrs	—	Sleep Disruption Score avg 5.8 / 10	<6 hrs on 6 of 12 nights
Energy	4.7 / 10	stable	—	range 3–6
Stim Load	5.3 / 10	—	high-load days (≥7): 4 of 12	caffeine ≥250mg on 5 days

MEDICATION SIGNAL

Escitalopram 10mg logged on 11 of 12 check-in days (one missed dose on day 7). Dose timing varied between 7:15 AM and 11:50 AM across logged days, with a standard deviation of 78 minutes — timing variability exceeds the 60-minute consistency threshold. No PRN or stimulant medications recorded for this period.

ADVANCED DATA (12 OF 14 DAYS POPULATED)

Exercise logged on 5 of 12 days (avg 28 min on logged days). Alcohol units averaged 1.4 across all days, 2.6 on drinking days (5 of 12). Social quality averaged 5.2 / 10. Workload friction averaged 7.1 / 10, with sustained scores above 7 across the first 8 days of the period. Hydration logged adequate on 6 of 12 days. Coping activities (breathing, meditation, movement) logged on 5 of 12 days.

QUALITATIVE THEMES

Journal entries from 9 of 12 days reference work. Recurring subjects include a specific project deadline, two named colleagues, and language clustered around *behind*, *catching up*, and *too much*. Three entries reference difficulty falling asleep specifically on Sunday and Monday nights. No coping activities logged on 7 of 12 days.

Mode C — P-2284 (continued)

SYMPTOM PATTERNS

Headache. Reported on 5 of 12 days. Co-occurring signals on symptom days: sleep_disruption_score elevated (avg 7.4 vs. 4.6 on non-symptom days, $\Delta = 2.8$, $n = 5$); Stim Load elevated (avg 6.6 vs. 4.4, $\Delta = 2.2$, $n = 5$). No medication change events within ± 14 days of first symptom entry.

No other symptoms meet the ≥ 3 -day reporting threshold for this period.

FLAGS

Sleep Disruption — period average 5.8 / 10 exceeds the 5.0 flag threshold; six of twelve nights logged below 6 hours.

Headache Symptom Pattern — symptom meets reporting threshold (≥ 3 days) with quantitatively distinct co-occurring sleep and stim-load patterns on symptom days.

Sustained Workload Friction — scores ≥ 7 sustained for 8 consecutive days, coinciding temporally with the rising-stress trend.

SUGGESTED DISCUSSION TOPICS

1. The Sunday/Monday sleep difficulty pattern combined with the sustained workload friction — worth clarifying with the patient whether the sleep difficulty is anticipatory of the week ahead or carryover from the prior week.
2. The headache pattern's co-occurrence with sleep disruption and elevated Stim Load on the same days — worth confirming the patient's caffeine intake pattern and whether anything has shifted in dose timing or amount within the window.
3. Alcohol logged on 5 of 12 days at an average of 2.6 units on drinking days. Volume sits below clinical concern threshold and is being raised only because it co-occurs temporally with the disrupted-sleep pattern.

Methodology note. This summary was generated from 12 check-ins logged across a 14-day window. Pattern claims requiring ≥ 7 logged days are valid for this period. Statistical trend references are reported only where $N \geq 21$ with $p \leq 0.05$ and $R^2 \geq 0.25$; where the period contains insufficient data for statistical claims, the system reports observed averages and ranges without inferring trend. All scores are computed deterministically in code prior to any AI-generated language. Output is post-processed against a forbidden-language list to prevent diagnostic phrasing, medication recommendations, and certainty overclaims.

This document is a sample of system output for a hypothetical patient. No real patient data has been used. Numbers, journal references, and dates have been fabricated for demonstration.